

Policy on Safe and Secure Environment

«Hospital Name»

Document control

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Date created	_____	Date of implementation	_____
Prepared by	Engineering In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

The organisation has a system in place to provide a safe and secure environment.

1. Purpose

This policy sets out how «Hospital Name» keeps the facility safe and secure. It covers:

- Installing and inspecting patient-safety devices and infrastructure
- Providing accessible facilities for differently-abled persons
- Conducting monthly safety-inspection rounds
- Documenting inspection findings and acting on them
- Carrying out risk assessments before any construction, renovation or expansion of the facility

This policy covers safe and secure environment specifically. Related duties — like patient assessment, clinical care, medication management, patient rights, infection control, quality and safety monitoring, or hospital governance — are covered in the hospital's other policies, not repeated here.

Words marked like this are defaults. A blank marked _____ must be filled before issue.

2. Scope

This policy applies to «Hospital Name» staff involved in facilities, utilities, medical gases, and fire or non-fire emergencies, including:

- Engineering, biomedical, nursing and departmental leaders
- The Engineering In-Charge

- The Medical Superintendent
- Departmental leaders
- The Quality Coordinator

This policy covers safe and secure environment specifically. Related duties are covered in the hospital's other policies, not repeated here, including:

- Patient assessment
- Clinical care
- Medication management
- Patient rights
- Infection control
- Quality and safety monitoring
- Hospital governance

3. Policy standards

Patient-safety devices and infrastructure are installed and periodically inspected across «Hospital Name». Accessibility facilities meet regulatory minimums for differently-abled persons. Monthly facility-inspection rounds identify and monitor safety, security-risk and restricted areas. Every finding is documented, acted on and reviewed by the safety committee. No construction, renovation or expansion begins without a completed risk assessment covering noise, vibration and infection prevention and control.

Staff follow written guidance and keep the records listed in the traceability table.

4. Non-negotiable rules

1. Do not run a patient-care area that lacks required patient-safety devices (grab bars, bed rails, call bells, alarms, warning signs and fire-safety devices as applicable to that area) or for which no periodic inspection record exists.
2. Do not operate without providing at minimum the accessibility facilities for differently-abled persons that applicable regulatory requirements mandate — a wheelchair-accessible entrance and an adapted toilet at minimum.
3. Do not let a calendar month pass without a completed, checklist-based facility-inspection round; a round without a completed checklist is not a round for this purpose.
4. Do not leave a finding from a facility inspection round without a documented corrective and preventive action and a safety-committee review within the same calendar month.
5. Do not start construction, renovation or expansion of the existing hospital without a completed risk assessment covering noise, vibration and infection prevention and control in place before work begins.
6. Staff who see a FMS.1 rule broken report it the same shift to the Engineering In-Charge or the Medical Superintendent.

5. What we do

5.1 Patient safety devices and infrastructure are installed across the organisation and inspected periodically.

Patient-safety devices and infrastructure (grab bars, bed rails, stretcher and wheelchair belts, call bells, alarms, radiation or biohazard warning signs, fire-safety devices as listed for each area) are installed across «Hospital Name» and inspected at a defined interval (default monthly).

The Engineering In-Charge holds the current list and last inspection. A missing device in a care area that is in use is escalated the same shift.

5.2 The organisation has facilities for the differently-abled

«Hospital Name» has facilities for the differently-abled (at a minimum a wheelchair-accessible entrance and an adapted toilet, as regulatory requirement and this hospital's building allow).

The Engineering In-Charge keeps the current access list. A step-only entrance with no documented alternative does not satisfy this element.

5.3 Facility inspection rounds to ensure safety are conducted at least once a month.

Facility inspection rounds to ensure safety are conducted at least once a month using a checklist. Potential safety and security-risk / restricted areas are identified and monitored.

The Engineering In-Charge owns the round calendar. A month without a completed round is a gap.

5.4 Inspection reports of facility rounds are documented and corrective and preventive measures are undertaken.

Inspection reports of facility rounds are documented. Corrective and preventive measures are undertaken. The safety committee reviews the reports monthly. Pre- and post-correction evidence is kept for at least one accreditation cycle.

The Quality Coordinator files the reviewed reports with the Engineering In-Charge.

5.5 Before construction, renovation and expansion of the existing hospital, risk-assessment is carried out.

Before construction, renovation or expansion of the existing hospital, a risk-assessment is carried out covering noise, vibration and infection prevention and control. Construction-infection controls are covered in other hospital policies; this element is that the assessment happens before work starts.

The Engineering In-Charge holds the dated assessment. Work started with no assessment does not satisfy this element.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that FMS.1 is resourced and followed.
Engineering In-Charge	Owns day-to-day implementation and records for this standard.
Quality Coordinator	Audits this policy quarterly; holds training acknowledgements.
departmental leaders	Run the department-level duties this standard names.

7. Quality monitoring (RCA → CAPA)

The Quality Coordinator audits this policy quarterly. The audit reviews:

- Records for a sample of this standard's objective elements, checked against the What-we-do steps.
- CORE objective elements show no critical gaps in the sample.
- Stop-work events (if any) are logged with outcome.

Root-cause analysis is required when a gap found in this audit remains open beyond 90 days.

This policy is reviewed annually, and sooner after a related facility change, utility failure, equipment recall or fire-plan change.

8. Training and staff acknowledgement

Staff covered by this policy are trained at induction and once a year after that. Training covers the What-we-do steps, non-negotiables and stop-work (if present).

Staff acknowledgement

I have read the Policy on Safe and Secure Environment of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

9. Distribution

This policy is distributed to:

- Medical Superintendent
- Engineering In-Charge
- Quality Coordinator
- departmental leaders
- staff covered by FMS.1

10. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
CORE	Core objective element (NABH)
FMS	Facility Management and Safety (NABH Hospitals 6th Edition chapter)
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element

11. Traceability to NABH HCO Full Accreditation 6th Edition FMS.1

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
FMS.1.a	CORE	Patient safety devices and infrastructure are installed across the organisation and inspected periodically.	Section 3; 5.1	Engineering In-Charge
FMS.1.b	Commitment	The organisation has facilities for the differently-abled.	Section 3; 5.2	Engineering In-Charge
FMS.1.c	CORE	Facility inspection rounds to ensure safety are conducted at least once a month.	Section 3; 5.3	Engineering In-Charge
FMS.1.d	Commitment	Inspection reports of	Section 3; 5.4	Engineering In-Charge

		facility rounds are documented and corrective and preventive measures are undertaken.		
FMS.1.e	Commitment	Before construction, renovation and expansion of the existing hospital, risk-assessment is carried out.	Section 3; 5.5	Engineering In-Charge

12. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

FMS.1.a — Patient safety devices and infrastructure are installed across the organisation and inspected periodically.

- Current inventory of patient-safety devices and infrastructure (grab bars, bed rails, call bells, fire-safety devices) by area.
- Inspection log showing the last-inspection date for each device or area.
- Escalation record for any missing device found in an in-use care area.

FMS.1.b — The organisation has facilities for the differently-abled.

- Facility list showing the wheelchair-accessible entrance and toilet location(s).
- Regulatory-requirement reference for the accessibility provision made.
- Documented alternative arrangement for any area with step-only access.

FMS.1.c — Facility inspection rounds to ensure safety are conducted at least once a month.

- Monthly facility-inspection-round checklist, completed and dated.
- Round calendar showing no missed month.
- List of identified safety, security-risk or restricted areas being monitored.

FMS.1.d — Inspection reports of facility rounds are documented and corrective and preventive measures are undertaken.

- Documented inspection reports of facility rounds.
- Corrective and preventive action record for each finding.
- Safety-committee review record of the reports.

FMS.1.e — Before construction, renovation and expansion of the existing hospital, risk-assessment is carried out.

- Dated risk-assessment record covering noise, vibration and infection prevention, completed before work started.
- Engineering In-Charge sign-off before construction, renovation or expansion began.
- Cross-reference to the construction-infection-control measures applied alongside (covered in other hospital policies).

13. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Accreditation Standards for Hospitals, 6th Edition (January 2025) — Facility Management and Safety, standard FMS.1.
- NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — FMS.1 interpretations.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

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«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

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